



MADISON COUNTY MEMORIAL HOSP.
224 NW CRANE AVE, MADISON, FL 32340 - 850-973-2271

Patient Name: KATHLEEN WESTERMAN

DOB: 01/28/1961

MRN: 89523

Account #: 1073273

Note Type: ED Physician Note

All Demographics									
Patient Name	DOB	Age	Sex	Visit Number	Admission Date/Time	Attending Physician	Date of Service	Room and Bed	Emergency Contact
WESTERMAN, KATHLEEN M	01/28/1961	65 years	Female	1073273	07/11/2026 22:31	DANIEL KUDER	07/11/2026	ED-T1	

07/12/2026 00:29

History of Present Illness

Chief Complaint: **Chief Complaint:** LACERATION RIGHT LEG

HPI:

65-year-old female presents to the emergency department with complaints of large laceration to the right lower leg just prior to arrival. Reports she was walking in her camp round and tripped over a fire ring.

Denies weakness or decreased sensation distally or any other associated injuries.

Past Medical/Surgical/Family/Social History

Past Medical History:

All Problems						
Problem	ICD Code	Onset Date	Resolved Date	Age	Status	Comment
Laceration without foreign body, right lower leg, initial encounter	S81.811A				Active	

Past Surgical History:

Surgery List: No Surgical History Available

Past Family History:

Family History List: No Family History Available

Social History:

Smoking Status List: No Social History Available



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Allergy List

No Known Drug Allergies, Medication

Home Meds: Dose and Freq: No Home Medications Available

Review of Systems

ROS Negative Except as Noted in HPI

X	Yes
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Physical Exam

Vital Signs: This Visit																							
Date/Time	BP (mm/Hg)	BP Position/ Site	MAP (mm/Hg)	Heart Rate	Pulse Site	Resp	Temp (°C)	Temp (°F)	SPO2%	O2 L/min	FiO2	EtCO2 (mm/Hg)	O2 Device	Blood Sugar	Pain Score	Height (cm)	Height (in)	Weight (kg)	Weight (lbs/ ozs)	Scale	BMI	BSA	Head Cir (cm)
07/11/2026 22:34	164/91		115	91		18	36.5 Oral	97.7 Oral	98 %				Room Air 21%			165.1 cm	65 in	74.39 kg	164.0		27.29	1.85	

GENERAL: Well appearing and well nourished. In no apparent distress.

HEAD: Normocephalic, atraumatic.

EYES: Pupils are equal, round and reactive to light. Extraocular movement intact. Without injection or icterus.

EARS: Normal external exam without injury

MOUTH/THROAT: Airway is patent. Moist mucous membranes.

NECK: Supple, without masses. Trachea midline

CHEST/LUNGS: Full and equal chest rise and fall. Unlabored respirations.

Cardiovascular: Strong and equal pulses b/l. No lower extremity edema.

MUSCULOSKELETAL: Normal ROM, No deformity.

SKIN: No rashes. Warm without jaundice or cyanosis. Large flap laceration to the anterior aspect of the right lower leg with organic material noted.

NEUROLOGIC: GCS 15, No focal weakness. Normal gait

PSYCHIATRIC: Mood and affect are appropriate for age. No SI

ED Course



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Note Type: ED Physician Note

00:30

Laceration Repair

Indication: Laceration to Right lower leg

Laceration length: 22 cm

flap laceraton

Procedure

Verbal consent obtained

Wound prepped with hibiclens and draped.

Anesthesia with Lidocaine 1% with epi

Wound copiously irrigated with NS pressure wash removing a moderate amount of organic material from the wound

Wound explored through FROM with no foreign bodies or injury to deeper structures noted

Wound closed with Ethilon 4-0 #11 horizontal mattress technique

Antibiotic ointment applied and return precautions discussed with patient who will return here for suture removal

Pt tolerated procedure well.

Abnormal Results: Last 24 Hours: No Labs Available

Lab Results: Last 24 Hours: No Labs Available

Ordered Meds List

NEO/ BAC/ POLY PACKET [0.5 GM], X1

Radiology:



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Note Type: ED Physician Note

Assessment

The patient is well appearing, and nontoxic. After careful history, physical exam, vital signs, and review of any laboratory or diagnostic data as ordered, I made the decision the patient does not appear to have a life threatening illness and can be carefully discharged with close follow-up. I do not feel any further imaging or laboratory data is needed at this time. I have considered both emergent, urgent disease pathology, and the current DIFFERENTIAL DIAGNOSIS including but not limited to:

Fracture
N/V injury
foreign body
Laceration

Chronic conditions addressed impacting care:

I have given written and verbal discharge instructions.

Patient advised of the importance of following up with their PCP concerning findings, treatment, and status post treatment. Extensive conversation with patient regarding findings, differential diagnosis, return precautions and treatment plan. Patient was educated on any prescriptions and proper usage as well as the importance of following up with primary care in a timely fashion for ongoing health and wellness management. The nursing staff has reiterated the disposition plan as well.

Diagnosis Today

Problem: Laceration without foreign body, right lower leg, initial encounter, **ICD Code:** S81.811A, **Status:** Active,
Addressed Date: 07/12/2026

Plan

Keep wound clean and bandaged with antibiotic ointment until healed
Return in 10 days for suture removal
Cleanse wound twice daily with soap and water then reapply antibiotic ointment and bandage.
Take Tylenol over the counter as directed for pain
elevate your lower leg above the knee to reduce swelling and pain
Return immediately for any unexpected change in your condition
Thank you for choosing our Emergency Department for your healthcare! Please take your medicines prescribed as directed and be assured that you follow up with the physician provided or your PCP in the next 1-2 days to assure you are improving. All medical problems cannot be reasonably diagnosed in your ED visit today. Return for any changes



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Note Type: ED Physician Note

or concerns, including if your condition does not improve or you are unable to obtain follow-up. Some final results, including radiology and laboratory reports, do not return the same day, but are available on the patient portal or can be obtained through your PCP.

Medications given this visit:

Ordered & Completed Meds Table					
Ordered Medication	Start Date/Time	Dosage	Route	Frequency	Status
VAC- TETANUS DIPH TOX ADULT INJ	07/12/2026	1 DOSE	IM OPTIONS	X1	completed
NEO/ BAC/ POLY PACKET [0.5 GM]	07/12/2026	1 PACKET	TOPICAL	X1	active

Current Medications:

Today: No Discharge Medications Available

Discharge Medications: No Discharge Medications Available



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Note Type: ED Nurse Triage

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WESTERMAN, KATHLEEN M	01/28/1961	65 years	Female	1073273	07/11/2026 22:31	DANIEL KUDER	07/11/2026	H1	

07/11/2026 22:34

Triage

Mode of Arrival: POV

Triage Location: Room 3

Triage Acuity / Emergency Severity Index: 4

Presenting Problem(s) / Complaint(s): Laceration

Onset of Symptoms / Time of Injury: x30 minutes PTA

Therapies Initiated Prior to Arrival: Bandaged

Pain Assessment		
Pain Scale	Location	Quality
3/10	RLE	Throbbing

Vaccines					
Influenza		Pneumonia		Tetanus	Childhood Immunizations
☐	Current for this season	☐	Current / Within 5 years	☐	X Up to date
☐	Not current	☐	Not current	X	Not up to date

Vital Signs: This Visit																			
Date/Time	BP (mm/Hg)	BP Position/ Site	MAP (mm/Hg)	Heart Rate	Pulse Site	Resp	Temp (°C)	Temp (°F)	SPO2%	O2 L/min	FI02	EtCO2 (mm/Hg)	O2 Device	Blood Sugar	Pain Score	Height (cm)	Height (in)	Weight (kg)	Weight (lbs/ ozs)
07/11/2026 22:34	164/91		115	91		18	36.5 Oral	97.7 Oral	98 %				Room Air 21%			165.1 cm	65 in	74.39 kg	164.0

Allergy Table		
Allergen	Type	Reaction
No Known Drug Allergies	Medication	

General Appearance				
X	No acute signs of distress		Appears ill	Appears toxic
X	Respirations unlabored		Mild distress	Moderate to severe distress



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Note Type: ED Nurse Triage

Home Meds: Dose and Freq		
Medication	Dosage	Frequency
No Home Medications		

Initial Nursing Assessment

Time of Assessment: 2234

Narrative Notes
65 y/o female presents to the ED via POV with a chief complaint of laceration. Pt ambulated to ED rm 3 where vitals were obtained and recorded. Pt reports that approximately 30 minutes PTA, she was walking when she tripped and hit her RLE on an outdoor firepit causing a laceration to her RLE. Pt denies any other injury from the fall. Pt rates pain 3/10 describing it as throbbing. Pt denies taking any medications prior to arrival. Pt states that EMS responded to her initially, bandaged her leg then she came in POV to the ER. Pt denies UTD TD vaccine. Pt A&O x4, shows NAD with even and unlabored breathing. Provider aware. Side rails x2, call light within reach.

ED Laceration

Location: RLE - noted to her anterior shin

Length: 22 cm

Distal NVT: Neuro intact Vasc intact - pt has FROM noted to her R foot and ankle

Depth/shape/contamination: Flap Irregular Contaminated mildly Wound with dirt



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Note Type: ED Nurses Note

Pt to ED trauma bed 1 via stretcher for provider assessment. Pt lying in bed appearing in NAD with even and unlabored respirations. Side rails x2. Call light in reach. Pt able to make needs known.



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Note Type: ED Nurses Note

Pt ambulated without difficulty to ED room 4 for provider assessment.



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Note Type: ED Nurse Disposition Note

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07/12/2026 00:44

DISPOSITION

DISCHARGE

Discharge To: Home

Discharge Instructions Given to: Patient

Condition: GOOD

Verbalizes understanding of instructions given: YES

Prescriptions Given to: None

Time of Departure: 0044

Mode of Departure: POV

Accompanied by: Family

TAO, telfa, and rolled gauze applied to laceration site. Pt tolerated well. Pt ambulated to lobby with slight limp appearing in NAD with even and unlabored respirations.

Diagnosis Today				
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Note Type: ED Discharge Instructions

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07/12/2026 00:33

Chief Complaint: LACERATION RIGHT LEG

Diagnosis:

Diagnosis Today				
Problem	ICD Code	Status	Comment	Addressed Date
Laceration without foreign body, right lower leg, initial encounter	S81.811A	Active		07/12/2026

Additional Instructions:

Keep wound clean and bandaged with antibiotic ointment until healed

Return in 10 days for suture removal

Cleanse wound twice daily with soap and water then reapply antibiotic ointment and bandage.

Take Tylenol over the counter as directed for pain

Elevate your lower leg above the knee to reduce swelling and pain

Return immediately for any unexpected change in your condition

Thank you for choosing our Emergency Department for your healthcare! Please take your medicines prescribed as directed and be assured that you follow up with the physician provided or your PCP in the next 1-2 days to assure you are improving. All medical problems cannot be reasonably diagnosed in your ED visit today. Return for any changes or concerns, including if your condition does not improve or you are unable to obtain follow-up. Some final results, including radiology and laboratory reports, do not return the same day, but are available on the patient portal or can be obtained through your PCP.

Medications administered during this visit:



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Note Type: ED Discharge Instructions

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NEO/ BAC/ POLY PACKET [0.5 GM]	07/12/2026	1 PACKET	TOPICAL	X1	active

New Scripts:

Discharge Medications: No Discharge Medications Available

Follow-up with your Healthcare Provider as directed.

Return to the Emergency Department if symptoms persist or worsen.

Nurse

Patient / Guardian